Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care

Regulation

Answer:

I believe that AI in healthcare should be very close to unregulated from the federal government. For decades, the government has put a fear in institutions and the US population thinking that if private companies get access to their data that they will do harm to them which is very inaccurate. The only form of regulation that should be pushed is the evaluation of AI models and how well they can improve the clinical setting. Healthcare systems and plans should be incentivized and be prompted to use AI in the clinical setting as it will help improve patient outcomes, reduce cost, and improve access to care for patients in rural areas. Data should be shared with private companies so models can improve according to their patient population. The data should be stored and accessed through a secure network to a select number of companies however it should not be held back. Holding back data is what partially inhibits the use of AI in the clinical setting. When AI products get evaluated, they might not be up to par with what clinicians want the tool to be so they end up not procuring the product and deploying it to the clinical work environment. I believe health systems should be told that they must use AI products in the clinical setting within the next 3 years to push adoption but to also let them know that the cost of AI will be greater in the short term but beneficial in the long run.

Reimbursement

Answer:

As a strong proponent of AI adoption in clinical settings, I believe HHS has a critical opportunity to modernize payment policies that will unlock transformative improvements in patient care, outcomes, and system efficiency. The current fee-for-service framework's inherent rigidity and slow adaptation to innovation creates barriers that prevent healthcare providers from accessing and implementing high-value AI tools that could enhance diagnostic accuracy, personalize treatment plans, reduce medical errors, and ultimately save lives. By reforming reimbursement structures to incentivize value-based care and creating clear, expedited pathways for coverage of validated AI clinical interventions, HHS can ensure that payment policies actively promote—rather than hinder—the integration of cutting-edge technologies that improve patient outcomes while controlling costs. I urge HHS to establish flexible, outcomes-oriented payment models that reward providers for utilizing AI tools that demonstrably enhance care quality, create transparent evaluation criteria for AI clinical applications that enable faster coverage decisions, and foster a competitive marketplace where innovation thrives and patients benefit from rapid access to the most effective AI-powered diagnostic and therapeutic solutions. The healthcare system's future depends on our ability to embrace these technologies now, and payment policy reform is the essential catalyst for widespread, equitable AI adoption that will benefit all Americans.

Research & Development

Answer:

The most impactful investments HHS can make are in establishing clear, expedited approval frameworks for AI clinical tools, creating standardized data interoperability requirements that enable private sector innovation, and funding open-access datasets and benchmarking platforms that level the playing field for startups and established companies alike—while allowing the competitive market to drive development, deployment, and iteration at the speed healthcare demands. Have a quota that the department must meet per year for private and public sector companies to join forces and receive grants to prioritize research and commercialization. The amount of startups receiving the funding should be great amount and should align with HHS mission for creating Food as Medicine as a preventative approach to medicine and health outcomes. **Most of R&D funding has gone to non profits and academic institutions which yields poor results. There needs to be a strong alliance between the private sector and public sector with an accelerated approval process instead of waiting 9-12 months to know if you are approved.** Partner with security based companies to help companies create an encrypted infrastructure for easy deployment and receiving security certifications. This initiative should treat the R&D as a form of program for early stage companies that have a high probability of commercialization and should create a clear path to long term contracts for use in the clinical setting.

Additional Questions:

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

A: Use of patient data, funding/reimbursement, and the lack of proper process that healthcare systems should follow on evaluating AI models. The approval process for AI models is also extremely slow between 12-18 months before you can start any partnership which should not be the case if in the private sector.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

A: Every single act that has been implemented or in the process of approval including the Obesity Act should include the use of advanced technologies in the bill for reimbursement. The Personal Health Investment Act should include healthier lives and lifestyles by using advanced technologies like Agentic AI and medically tailored meals for that act.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (*e.g.,* relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (*e.g.,* contracts, grants, cooperative agreements, and/or prize competitions)?

5. How can HHS best support private sector activities (*e.g.,* accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

A: I believe HHS should have a **preferred list** of health plans and systems that would be more than willing to adopt AI products if we are a private company on the list of preferred AI products that HHS has approved and reviewed. If there is an accelerated pathway that HHS can create to have vendors and buyers adopt the technology, that can help improve the adoption, deployment and contract agreements between sectors. I also believe if we can standardize the process and have EMR systems adopt the changes, that would help with deployment, fitting into the clinical workflows. If there could be a standard contract agreement created for health systems, plans, and vendors, that can reduce the amount of attorney fees, back and forth between multiple departments, and accelerate adoption of devices.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

A: Some of the primary hurdles for AI in clinical care are that the smaller institutions do now have a process put in place to adopt in general. The smaller institutions do not know how to move forward with clinical AI, how to start or what policy they should follow. On the other hand, the larger institutions are overly bureaucratic in making decisions. Larger institutions tend to have several layers of red tap on adopting any form of clinical decision making for AI adoption, the expectations are incredibly high by wanting 7 figures in revenue with our customers, 18 months to make a decision because of the committees. If there can be an open department that is specifically used for AI adoption and evaluation throughout the entire healthcare system, that would decrease the amount of hurdles to go to market. They usually have people at the bottom of command try to send you to the next department and figure out who would be a good fit for the institution. I also believe that if Health Plans and Health Systems published the top 10 needs of their specific hospitals by using advanced technology just like Y Combinator does, that would be extremely helpful.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

A: I would say accelerated care and waitlists to nutritional coaching. At Kaiser Permanente, patients have said that getting into the nutrition programs are very difficult to get into, and the waitlist can take anywhere from 3-12 months. At that rate, you are closer to your deathbed than anything else. Getting into holistic preventative medicine is better for patient outcomes so investing heavily into nutritional devices, technologies should give healthcare reimbursement for providers to help incentivize them in using advanced technologies to reduce obesity, Heart Disease, Type 2 diabetes and pre diabetes.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

A: Food is Medicine! Lifestyle interventions should be focused on and should not be regulated if you are providing nutrition and exercise plans for patients to follow.

a. Are there published findings about the impact of adopted AI tools and their use in clinical care?

https://www.sciencedirect.com/science/article/pii/S0735109723083171

https://pubmed.ncbi.nlm.nih.gov/35129822/

https://www.tandfonline.com/doi/10.1080/03007995.2020.1787971?url_ver=Z39.88-2003&rfr_id=ori:rid:crossref.org&rfr_dat=cr_pub%20%200pubmed#abstract

b. How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?